

Policy of Direct Admission to Paediatric Services at the Royal Hospital

Policy purpose:

This policy is made to organise and facilitate safe patient transfer between two different institutes without compromising emergency department (ED) load and work dynamic.

Scope

This policy applies to all paediatric Health care facility and includes:

- 1- Receiving of patients between one Ministry of Health (MOH) care facility to Child Health department or paediatric surgical department at the Royal Hospital.
- 2- Receiving of patients from one Private Health establishments (PHEs) to child health department or paediatric surgical department at the Royal Hospital.

Definitions:

- 1- Direct admission: is a process whereby patients are admitted from other hospitals (including: MoH Health Care Facilities, Non-MoH Government Health Care Facilities and Private Health Establishments) directly to Royal Hospital inpatient unit rather than via the hospital's ED. The process is coordinated between the referring clinician, the receiving consultant, the Patient Flow Coordinator (bed manager) and the transporting clinician.
- 2- Referring clinician: the clinician who initiates (sending) the referral of patient.
- 3- Receiving consultant: The consultant who accepted the referral.
- 4- Patient Flow Coordinator (bed manager) : A staff responsible for organizing patients flow and allocating beds for admitted patients within different department inside the hospital and from other institutes
- 5- Transporting clinician: Clinician responsible for transferring a patient between two different institutes.
- 6- MoH Health Care Facilities: These are institutions that belong to Ministry of Health and basically categorized into three levels based on type of health care provided which include: primary, secondary, and tertiary healthcare. And according to the nature of healthcare that are provided, these institutions are given the following names: Health Centre, Extended/Polyclinic, Local/Wilayat Hospital, Regional Referral Hospital and Tertiary Hospitals.
- 7- Non-MoH Government Health Care Facilities: These are institutions belong the Government other than MoH healthcare facilities; like Armed Force, Royal Oman Police Hospital and Royal Court Hospitals and Polyclinics.
- 8- Private Health Establishments: all Private hospitals registered in MOH
- 9- Routine Referral: Is the referral in which the condition of the patient is not urgent in character but needs referral for second opinion, higher level of investigations and for seeking routine management / admission.
- 10- Urgent Referral: Is the referral in which the condition of the patient needs appointment and consultation within 72 Hours of the time of request to prevent impending complication.

Overview

Ministry of Health is committed to provide the community with timely and equitable access to health services and ensuring the efficient use of available resources in public hospitals.

Historically there has been an over-reliance on the Emergency Department to manage the admission of many patients, including transfers from other sites, where a medical review has already occurred. Consequently, some patients receive multiple reviews which may result in treatment delays and contribute to ED overcrowding.

The Direct Admission to a Hospital Inpatient Unit Policy enhances patient care by establishing a streamlined process for the management of safe and timely hospital admission for those patients considered suitable to be directly admitted to a hospital inpatient unit rather than via the ED.

Clear pathways support streamlined transfer of care processes. Recognition and response to clinical deterioration requires effective communication, documentation, and multi-disciplinary teamwork.

Rational:

Increasing ER volumes, long waiting times, and rising ED costs speak to the importance of better understanding hospital admission alternatives and the associated risks and benefits. Encouraging more direct admissions is among the recommended solutions, but evidence to guide best practices must be in place.

Policy statement

1- Roles and Responsibilities:

A. Referring clinicians are responsible for :

- Identifying patients requiring transfer to another hospital for inpatient admission.
- Initiating the admission process.
- Informing patients of the admission process and admission details.
- Arranging an appropriate mode of transport as required.
- Providing a comprehensive clinical handover, including expected date and time of arrival.
- Ensuring that a transfer form is filled stating that the patient is fit for direct admission.
- Notifying the requesting hospital of any unexpected delays in transport (**inform patient flow coordinator**).
- Monitoring the patient's clinical condition during transfer and alerting the receiving hospital to any clinical deterioration that may impact on the patient's eligibility for direct admission and the need for emergency intervention.
- Escalating any significant issues to the relevant senior clinician, Director, or Head of Unit.

B. Transferring (escorting) physician is responsible for:

- Coordinating requests from referring clinicians to transfer patients for admission.
- Notifying the requesting hospital of any unexpected delays in transport.
- Providing timely clinical handover to receiving clinicians.
- Monitoring the patient's clinical condition during transfer and alerting the receiving hospital to any clinical deterioration as this may impact on the patient's eligibility for direct admission.

C. Receiving Consultant is responsible for:

- Considering the clinical need of the request for admission.
- Determining whether the patient can be admitted directly to the inpatient unit or whether the patient's clinical condition requires ED assessment when a request for admission has been accepted.
- Ensuring that all patient information, including expected date and time of arrival, is communicated to relevant inpatient team members.
- Ensuring that acceptance form to be filled, including all relevant details and to be sent and communicated to PER consultant in case PER care is expected for the patient on arrival.
- Ensuring that patients received via direct admission receive treatment in a safe and timely manner.
- Escalating any significant issues to the relevant Director or Head of Unit.

****The receiving Consultant** retains responsibility and accountability for clinical decision-making regarding patient transfer and admission requests.

**** The receiving Consultant** should consider the following factors in determining whether the patient is suitable for admission:

- 1- The patient's clinical condition
- 2- Urgency
- 3- Clinical actions required on arrival or during admission with collaboration of the transferring physician.
- 4- Any other treatments and referrals that may be required.

D. Patient Flow Coordinator is responsible for:

- Managing patient flow and effective bed utilisation in accordance with this Policy Guideline.
- Escalating any issues as per local processes.
- Coordinating requests from referring clinicians to transfer patients eligible for direct admission.
- Informing the receiving team about any delay or change in the expected time of arrival.

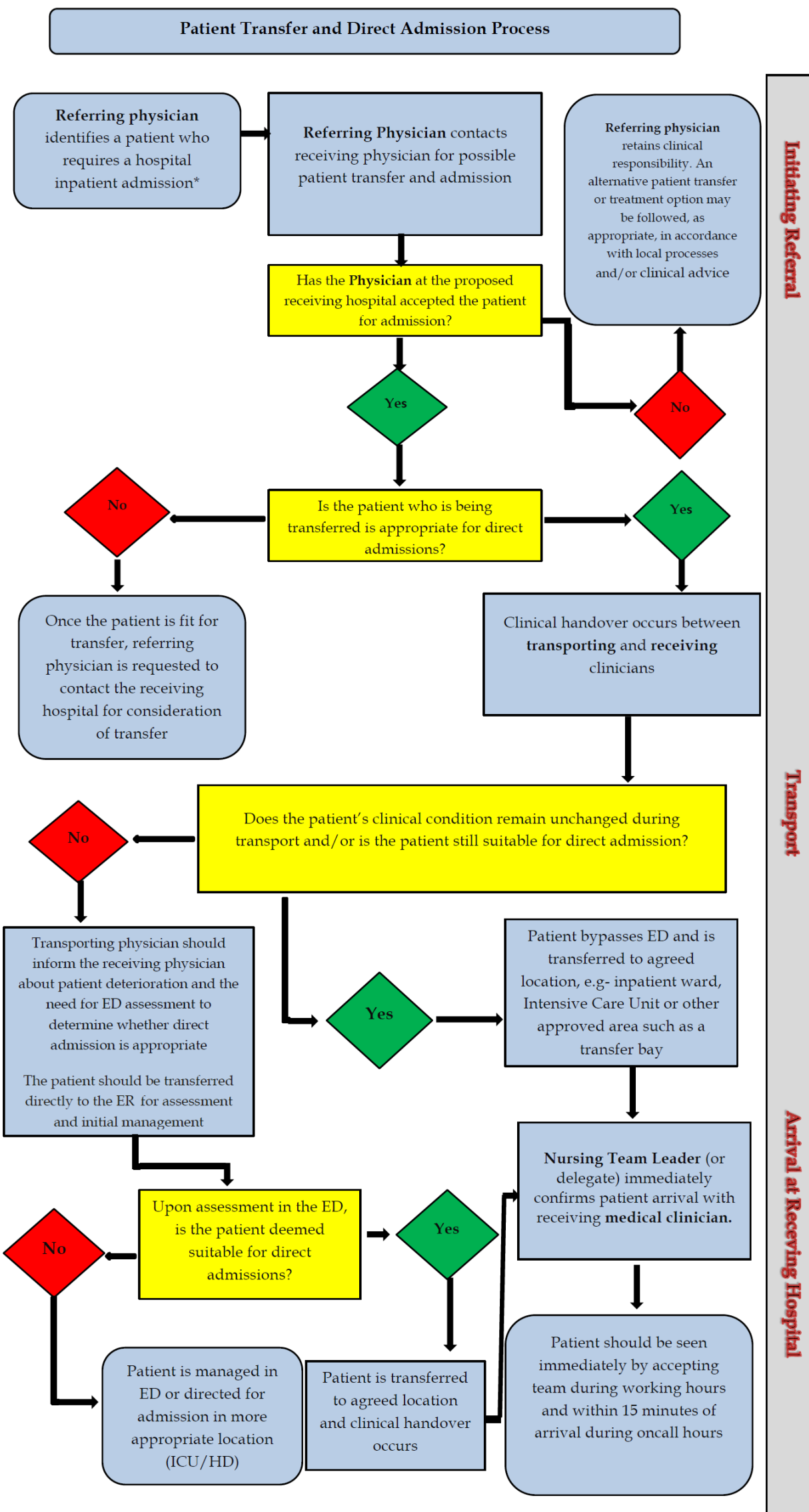
2- Patient arrivals

A- Patient suitable for direct admission on arrival.

- Patients whose clinical condition is unchanged or who remain suitable for direct admission on arrival should bypass ER and be transferred to the agreed designated location (treatment area). Transit lounges should be utilised, where available, to facilitate efficient turn-around times for ambulance crews.
- Once the patient arrives to allocated area, he should be seen immediately by accepting team during working hours and within 15 minutes of arrival by oncall team during oncall hours/weekends.
- These patients should bypass the ER unless the receiving Consultant has determined there is a clinical reason for presentation to the ER for assessment or investigation that cannot be provided in the receiving ward. Prior discussion between the receiving consultant and ER consultant should be ensured.

B- Patient's condition deteriorates enroute.

- If clinical condition deteriorates enroute and the patient is no longer considered suitable for direct admission, the receiving hospital should be notified as soon as possible that the patient requires urgent assessment on arrival.
- The transporting clinician should contact the receiving hospital's ER to advise that the patient has clinically deteriorated enroute and requires assessment to determine whether direct admission is still appropriate. The patient should be transferred directly to the ER or, where pre-arranged, to the appropriate Intensive Care Unit/specialist unit.



Appendix I

Patient’s Acceptance Form (To be filled by receiving team)

Child Health Department Patient Acceptance Form	
Date/Time of receiving call: ----- Date/Time of accepting the patient: ----- Referral Status: Accepted Awaited Rejected Bed Manager Informed Yes No (if yes pls provide name)----- Bed availability: Yes No	Referring Hospital: Referring Doctor: Receiving Team: Acute Ped - Oncall -Sub-Specialty Receiving Doctor: Discussed with Consultant: Who has been involved from subspeciality before theconsult?
Patient Information: Name----- Weight kg. Sex: M F Age: Royal I.D: (if present) Referring Hospital I.D:	Transfer from: Ward ER High dependency PICU NICU Accepted for admission to: Ward High dependency PICU Consultant Informed: Yes No ,Name: Urgency of transfer: Emergency Urgent Elective Reason for transfer: More specialized care Surgery & Intervention Lack of bed Radiology
Diagnosis: ----- Brief clinical details: ----- ----- Investigations results: ----- ----- Drugs On:----- ----- Vital Signs: HR: ----- BP: ----- RR----- SatO2 ----- Perfusion: ----- Reason for Consult:	
Advice given: 1- to call back for update before transfer if accepted 2- 3- 4-	
Updates: Date (/ /) (please fill this part if the patient will not be transferred on the same day of the consult)	

Bed Manager Notes/Signature: